3.6 Pulmonary Tuberculosis*

Assessment	Nursing Diagnosis	Plan / Expected Outcome	Nursing Interventions	Evaluation
Subjective • "Can I stop the tablets when I feel better?" • "I have had a cough for 2 months." **Objective** • Sputum smear positive for acid-fast bacilli • Anti-TB treatment recently started • Cannot name his medicines	**ACTUAL** Ineffective health self-management related to complexity of long-term treatment as evidenced by intention to discontinue early and inability to name medications.	• Before discharge, names his anti-TB drugs, their schedule and the length of treatment.	• Explain that treatment lasts at least 6 months and must not be stopped early. • Support directly observed therapy (DOT). • Teach the common side effects: orange urine with rifampicin, jaundice, visual changes, tingling. • Book follow-up sputum tests.	• Met when he explains the regimen correctly.
Subjective • "I have no appetite." **Objective** • Lost 5 kg • BMI 17.5 kg/m²	**ACTUAL** Inadequate nutritional intake related to anorexia and increased metabolic demand as evidenced by weight loss and low BMI.	• Gains weight steadily during treatment.	• Provide a high-energy, high-protein diet. • Give pyridoxine with isoniazid as prescribed. • Weigh weekly.	• Achieved as weight increases.
Subjective • Lives in a household of seven **Objective** • Smear-positive pulmonary TB • Overcrowded housing	**RISK** Risk for infection [transmission to others] as evidenced by risk factors: airborne transmission and close household contacts.	• No secondary cases; he keeps to airborne precautions.	• Keep him in airborne isolation until no longer infectious. • Teach cough etiquette and safe sputum disposal. • Encourage good ventilation at home. • Refer household contacts for screening.	• Outcome reached when precautions are followed and contacts have been screened.

* **References for this care plan:** [35], [37], [117], [119], [141]. Full entries are listed in the References section of the CPMN book.